

Medicaid coverage of obesity drugs: what it did and what it could cost

Erick Kiprotich Yegon · Epidemiologist and data scientist

Question. What did Medicaid coverage of Wegovy and Zepbound do to prescriptions, how many adults are eligible, and what could coverage cost a programme of 1,000,000 enrollees over five years?

Three findings

1. **Coverage raised prescriptions.** About 12.2 extra prescriptions per 1,000 enrollees per quarter (95% CI 8.4 to 16.0), rising from 1.3 in the first quarter to 17.2 after eight; ten covering states against 34 never-covering jurisdictions. A placebo shows 0.4 (95% CI -0.1 to 0.8).
2. **Many adults are eligible.** 128.8 million U.S. adults meet the FDA label criteria (lower bound; 95% CI 117.2 to 140.3 million), including 17.8 million with Medicaid (95% CI 14.7 to 20.9 million).
3. **The budget depends on the rebate.** Central five-year net cost \$161.3 million (\$2.69 per enrollee per month; rebate 51.2% assumed); scenario range \$55.6 to \$369.4 million; \$68.3 million at the announced \$245 price.

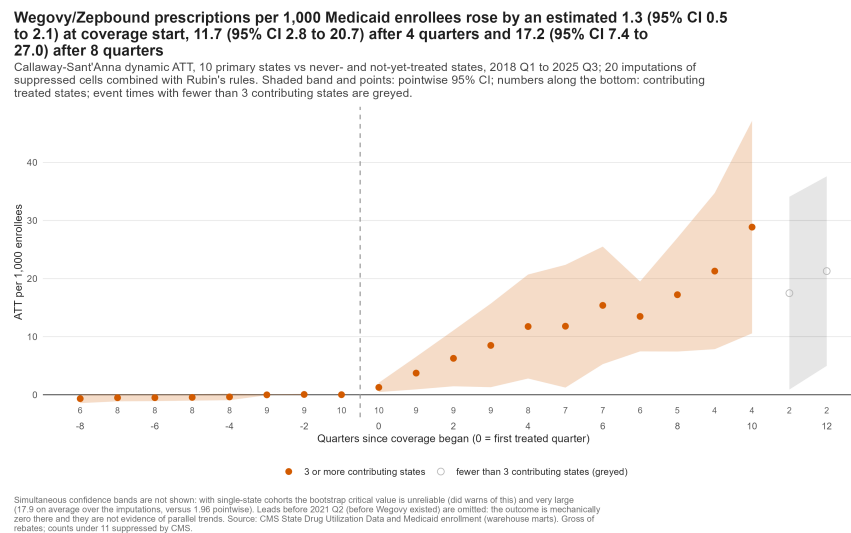


Figure 1: Coverage effect by quarter since coverage began (source: CMS State Drug Utilization Data and Medicaid enrollment; part of the growth is national market growth)

Methods. Public data (CMS Medicaid State Drug Utilization Data and enrollment, NHANES, MEPS, KFF polls); difference-in-differences for staggered adoption with imputed suppressed cells; survey-weighted eligibility; ISPOR-style budget impact model with probabilistic sensitivity analysis. Warehouse: PostgreSQL and dbt (54 models, 126 tests); analysis in R; Quarto and Shiny.

Skills. Real-world evidence · HEOR · market access · claims data · NDC coding · causal inference · budget impact · SQL · dbt · R · Quarto · Shiny.

Links. Code: github.com/erickyegon/incratin-access-value (to be published) · Report: [report/report.html](#) · Deck: [deck/deck.pdf](#).

Public aggregate data; no company affiliation or endorsement; not patient-level claims; gross of rebates unless stated; scenarios, not forecasts. I used AI tools to help write code and documentation. The study design, methods and conclusions are my own, and I verified all results.